

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Benzoyl Peroxide plus Clindamycin or Adapalene plus Benzoyl Peroxide for the treatment of Acne Vulgaris

Summary of NICE / NICE CKS Guidelines for Acne

Overview

- **Acne vulgaris** is a chronic inflammatory skin condition affecting the **pilosebaceous units**, typically on the **face, back, and chest**.
- Common in adolescents but can persist into adulthood.

Assessment

Types of Lesions

- **Comedones** (open = blackheads; closed = whiteheads)
- **Inflammatory lesions** (papules, pustules, nodules, cysts)
- Assess for:
 - o **Severity** (mild, moderate, severe)
 - o **Scarring** or **psychological distress**
 - o Duration and previous treatments

Red Flags – Consider Referral

- Severe nodulocystic acne
- Risk of or presence of **scarring**
- Significant **psychological impact**
- No response to ≥ 2 standard treatments over 6 months

Management – First-Line Treatment (Based on Severity)

Severity	Treatment	Duration
Mild acne	Topical retinoid (e.g. adapalene) + topical benzoyl	At least 12 weeks

Severity	Treatment	Duration
	peroxide	
Moderate acne	Topical retinoid + benzoyl peroxide , OR	
Topical antibiotic + benzoyl peroxide	Avoid monotherapy	
Severe or widespread acne	Oral antibiotic (e.g. doxycycline or lymecycline) + topical retinoid or benzoyl peroxide	Limit oral antibiotics to 12 weeks

Do **not** combine oral and topical antibiotics to avoid resistance.

Alternative and Specialist Options

- **Oral contraceptives** (for females with hormonal acne)
 - Consider **co-cyprindiol** if acne is unresponsive and other methods are unsuitable.
- **Isotretinoin** (oral retinoid): for **severe, scarring, or resistant acne** — **specialist only** due to teratogenicity and psychiatric risk.

General Advice and Skin Care

- Cleanse skin gently twice daily with a non-comedogenic cleanser.
- Avoid picking or squeezing lesions.
- Advise that treatment takes **at least 6–8 weeks** to show improvement.
- Use **non-comedogenic** skincare and cosmetics.

Follow-Up and Referral

- Review after 8–12 weeks to assess response.
- **Refer to dermatology** if:
 - Moderate-to-severe acne not responding to treatment
 - Scarring or pigmentation developing
 - Isotretinoin is being considered

Patient Group Direction

for the supply/administration of

Benzoyl peroxide plus Clindamycin

for the treatment of

Acne Vulgaris

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Benzoyl Peroxide plus Clindamycin is indicated for the topical treatment of mild to moderate acne vulgaris in individuals aged 12 years and over, particularly where comedones, papules and pustules are present.
Inclusion criteria	- Individuals aged 12 years and over. - Clinical diagnosis of mild to moderate acne vulgaris. - Informed consent obtained. - No contraindications to ingredients.
Exclusion criteria	- Known hypersensitivity to clindamycin, lincomycin, or benzoyl peroxide. - History of antibiotic-associated colitis. - Severe acne requiring systemic therapy. - Broken or inflamed skin at application site. - Safety in human pregnancy is not established. Should only be prescribed to pregnant women after careful risk/benefit assessment. Clindamycin has been found in breast milk so should only be used during breastfeeding if the expected benefit justifies the potential risk to the infant and should never be applied to the breast area to avoid accidental ingestion by the infant. For pregnancy and breastfeeding refer the patient to GP
Cautions	- Avoid contact with eyes, mouth, mucous membranes, or broken skin. In the case of accidental contact, rinse well with water. - Use sunscreen and limit sun exposure due to increased sensitivity. - Monitor for signs of skin irritation; discontinue if severe. - Use caution in patients with a history of gastrointestinal disease. - Use with caution in atopic patients. - Consider referral to dermatologist if acne is causing severe scarring, persistent pigmentary changes or is contributing to persistent psychological distress or a mental health disorder.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Benzoyl Peroxide plus Clindamycin 10mg/g + 30mg/g and 10mg/g + 50mg/g gel
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C) before dispensing. Once dispensed, store below 25°C and use within 2 months.
Route/method of administration	Topical application to affected area once daily in the evening. Should be applied in a thin film after washing gently with a mild cleanser and fully drying. If the gel does not rub into the skin easily, too much is being applied. Patients should be advised that excessive application will not improve

	efficacy but may increase the risk of skin irritation. If excessive dryness or peeling occurs, frequency of application should be reduced or application temporarily interrupted. Hands should be washed after application.
Dose and frequency	Apply a thin layer to the entire affected area once daily in the evening. A clinical decision must be made on the strength of gel to be used. The 10mg/g + 30mg/g strength is suitable for mild to moderate presentations and minimizes risk of skin reactions. Where the lower strength has proven less effective, for more moderate presentations or where a patient has previously tolerated 50mg/g (5%) benzoyl peroxide, the 10mg/g + 50mg/g may be preferred.
Quantity to be administered and/or supplied	Up to 1 x 30 g tube per treatment course.
Maximum or minimum treatment period	Maximum of 12 weeks continuous use; review required for repeat courses.
Adverse effects	Dryness, peeling, redness, burning, and irritation of the skin. Rare: diarrhoea, colitis due to clindamycin absorption. Refer to SmPC. Application site reactions including skin and hair discoloration.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Avoid over-cleaning acne-prone skin. Avoid oil-based comedogenic skin care products, make-up and sunscreens, and if make-up is used it must be removed at the end of the day. Persistent picking or scratching of lesions can increase the risk of scarring. Treatments may take 6-8 weeks to be effective and may irritate the skin, especially at the start of treatment.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date		Expiry date	
1/11/25		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

Patient Group Direction

for the supply/administration of

Adapalene plus Benzoyl Peroxide

for the treatment of

Acne Vulgaris

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adapalene and benzoyl peroxide gel is indicated for the topical treatment of mild to moderate acne vulgaris in individuals aged 12 years and over, especially where comedones and inflammatory lesions are present.
Inclusion criteria	- Individuals aged 12 years and over. - Diagnosis of mild to moderate acne vulgaris suitable for topical treatment. - Informed consent obtained. - No contraindications to adapalene or benzoyl peroxide.
Exclusion criteria	- Known hypersensitivity to adapalene, benzoyl peroxide, or any excipients. - Severe acne requiring systemic treatment. - Presence of eczema, broken or sunburned skin at the application site. - Pregnancy or planning pregnancy.
Cautions	- Avoid contact with eyes, mouth, mucous membranes, and broken skin. - Advise patient to use sunscreen and limit sun exposure. - Apply at night and wash hands after use. - Discontinue if excessive irritation or allergic reaction occurs. - If product enters the eye, wash immediately with warm water. - Caution should be exercised if cosmetics with irritant or drying effects are used, as they may produce additive irritant effects with the medicinal product. - Avoid contact with any coloured material including hair and dyed fabrics as this may result in bleaching and discoloration. - A decision must be made whether to discontinue breast-feeding weighing up the benefit of breast-feeding for the child against the benefit of therapy for the woman. To avoid contact exposure of the infant, application to the chest should be avoided if used during breast-feeding. A risk to the suckling child cannot be excluded.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Adapalene and benzoyl peroxide 0.1% / 2.5% and 0.3% / 2.5% gel
Legal category	POM
Storage	Store below 25°C. Do not freeze.

Route/method of administration	Topical application to affected area once daily in the evening.
Dose and frequency	Apply a thin layer to the entire affected area once daily in the evening to clean and dry skin.
Quantity to be administered and/or supplied	Up to 1 x 30 g tube or pump per treatment course.
Maximum or minimum treatment period	Initial treatment for up to 12 weeks; reassess if longer use is required. If no improvement is observed after 4-8 weeks of treatment, the benefit of continued treatment should be considered
Adverse effects	Skin irritation, dryness, redness, peeling, burning. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. To avoid over-cleaning the skin (which may cause dryness and irritation). Acne is not caused by poor hygiene.

	To use a non-alkaline (skin pH neutral or slightly acidic) cleansing product twice daily on acne-prone skin. To avoid oil-based skin care products, make-up and sunscreens, and if make-up is used to remove it at the end of the day. That persistent picking or scratching of lesions can increase the risk of scarring. That treatments may irritate the skin, especially at the start of treatment. To reduce the risk of skin irritation to start with alternate-day or short-contact application (for example washing off after an hour). That there is not enough evidence to support specific diets for treating acne, but that it is important to maintain a healthy diet.
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